جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : ضحى محمد محمود حسين
المحافظة : القاهره **الحي / المدينة :** البساتين **العنوان :** ش الجنينه
رقم البطاقة : 28309010116569 **رقم التليفون :** 01027979861 **تاريخ تسجيل الإستمارة :** 24-10-2026
العمر(تاريخ الميلاد) : 01-09-1983 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ضحى محمد محمود حسين
National ID: 28309010116569

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



ضحى
محمد محمود حسين محمد
١٧ ش جنينة على عابد
البساتين - القاهره

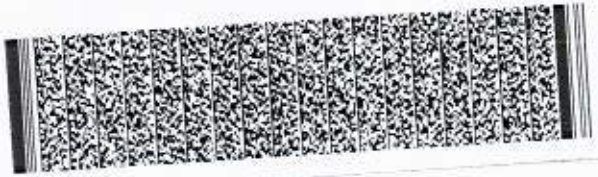
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٢٨٣٠٩٠١٠١١٦٥٦٩
حاصله على بكالوريوس اعلام ٢٠٢٥/٠٥

انثى مسلمة متزوجة
محمد عبدالسميع عبدالوهاب الدسوقي
البطاقة سارية حتى ٢٠٣٢/٠٥/١٢



DMS

كيماوى عكاكسول كاربوبلاتين

علاج اقاستين

عاجل

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Date Of Submission: Date Of MDT: 05/21/2026 12:28:33

General GRI-E MDT form

Patient Name :	DOHA محمد محمود حسين	Age :	43 Y
Sex :	○ Male ○ Female	Comorbidity :	-ve
ID No. :	28309010116569	Telephone :	01027979861
Family History :	-ve	Marital Status :	◉ Married ○ Divorced ○ Single
Menopausal Status :	10800	Reason of MDT :	
Menopausal Status :	MDT	MDT name::	Gynecological Cancers

Medical Report

Diagnosis :
Notes :

History:
In 2023: The patient resected Lt ovarian cyst, Dx as low grade microinvasive carcinoma.--> the Pt underwent total hysterectomy with bilat salpingo-oophorectomy, omentectomy and appendictomy.
Larger ovary showed few residual foci of microinvasive papillary serous carcinoma, low grade
Free cervix, uterine wall, other ovary, appendix, and omentum.

In May 2026: Left abdominal wall mass --> exploration was done 5/ 2026 exploration was done (a midline and vesical were excised Rt iliac lesion inseparable from from caecum and iliac vessels)--> positive for deposits of papillary malignant neoplasm.
For BRCA testing

Investigations:

Recommendations:
For Taxol/Carbo + Avastin

MDT DECISION:
For Pathology Revision and IHC at GRI-E lab.
CA125 and CEA
CT CAP e contrast (only performed MRI pelvis before exploration 3/2026)
if Confirmed Ovarian papillary serous carcinoma Recurrence to start Carboplatin -Taxol weekly +/- Avastine for 3-4 cycles then re-assess for 2nd look surgery possiblity
CGS test Including HRD

Signature	Hematology	Pediatric	Bone marrow transplant
dr nada hassan			



Name: ضحى محمد محمود حسين **MR. :** 10800	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

In 2023 the patient underwent resection of a left ovarian cyst that was diagnosed as low-grade microinvasive carcinoma. Subsequently, a total hysterectomy with bilateral salpingo-oophorectomy, omentectomy, and appendectomy was performed. Pathology of the larger ovary revealed a few residual foci of low-grade microinvasive papillary serous carcinoma; the cervix, uterine wall, contralateral ovary, appendix, and omentum were free of disease.

In May 2026 the patient presented with a left abdominal wall mass. Surgical exploration on 5 May 2026 included excision of a midline and vesical lesion as well as a right iliac lesion that was inseparable from the caecum and iliac vessels. Histology was positive for deposits of papillary malignant neoplasm. BRCA testing was ordered.

In April 2026 the patient was noted to have recurrent ovarian cancer, stage IV papillary serous carcinoma. The history included bilateral ovarian masses resected in November 2023 with microinvasive low-grade papillary serous carcinoma, and an anterior abdominal wall mass excised on 25 April 2026. Post-operative CT on 16 May 2026 demonstrated an ill-defined right deep pelvic soft-tissue mass (3.5 × 7.5 cm) infiltrating the sigmoid colon, a partially amalgamated soft-tissue lesion likely nodal and inseparable from the pelvic lesion, multiple enlarged right common iliac and right inguinal lymph nodes (largest 1.9 cm), and a right hypochondrial peritoneal lesion. The multidisciplinary team recommended a regimen of taxol, carboplatin, and avastin; BRCA1/2 results were pending. The patient began cycle 1 with taxol 290 mg, carboplatin AUC 5, and prophylactic filgrastim (weight 67 kg, height 170 cm, surface area 1.7 m²).

The patient received cycle 2 of taxol 290 mg, carboplatin AUC 5 with prophylactic filgrastim.

During cycle 3 the patient continued taxol 290 mg and carboplatin AUC 5 with prophylactic filgrastim, but reported grade 1–2 peripheral neuropathy, leading to a one-week postponement due to an absolute neutrophil count of 0.75 × 10⁹/L.

For cycle 3 the patient experienced grade 2 peripheral neuropathy; the regimen was adjusted to taxol 260 mg (10 % dose reduction) and carboplatin AUC 5 (10 % dose reduction). BRCA1/2 testing returned negative for mutation.

On 27 August 2026 the patient presented with a platelet count of 10 × 10⁹/L and received a platelet transfusion. A repeat CBC showed a platelet count of 47 × 10⁹/L; the chemotherapy was postponed for the week with plans to reassess the CBC the following week.

On 3 September 2026 the patient underwent CT imaging on 15 August at the center, which could not be directly compared to a prior scan at another institution for response assessment. Meanwhile, for cycle 4 week 1 the patient received taxol 60 mg/m² and carboplatin AUC 2; platelet counts were 68,

تحريرا في ,

التاريخ :	**18-09-2026**	**الوقت :**	

الطبيب المعالج : **Ali Adel**

يعتمد ,

47, and 10 × 10⁹/L on successive checks.

On 10 September 2026 a repeat CT on 15 August was compared with the 16 May scan, showing residual local mass and lymph nodes. Laboratory studies showed a platelet count of 55 × 10⁹/L; chemotherapy was postponed for the week with consideration of reducing carboplatin to AUC 1.5 the following week and administering epirubicin 10 mg daily for 24 hours.

The patient has completed cycle 4 week 1 of the taxol and carboplatin regimen.

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Date Of Submission: **Date Of MDT:** 05/21/2026 12:28:33

General GRI-E MDT form

Patient Name :	DOHA محمد محمود حسين	**Age :**	43 Y
Sex :	○ Male ○ Female	**Comorbidity :**	-ve
ID No. :	28309010116569	**Telephone :**	01027979861
Family History :	-ve	**Marital Status :**	◉ Married ○ Divorced ○ Single
Menopausal Status :	10800	**Reason of MDT :**	
Menopausal Status :	MDT	**MDT name::**	Gynecological Cancers

Medical Report

Diagnosis :
Notes :

History:	In 2023: The patient resected Lt ovarian cyst, Dx as low grade microinvasive carcinoma.--> the Pt underwent total hysterectomy with bilat salpingo-oophorectomy, omentectomy and appendictomy. Larger ovary showed few residual foci of microinvasive papillary serous carcinoma, low grade Free cervix, uterine wall, other ovary, appendix, and omentum. In May 2026: Left abdominal wall mass --> exploration was done 5/ 2026 exploration was done (a midline and vesical were excised Rt iliac lesion inseparable from from caecum and iliac vessels)--> positive for deposits of papillary malignant neoplasm. For BRCA testing
Investigations:	
Recommendations:	For Taxol/Carbo + Avastin
MDT DECISION:	For Pathology Revision and IHC at GRI-E lab. CA125 and CEA CT CAP e contrast (only performed MRI pelvis before exploration 3/2026) if Confirmed Ovarian papillary serous carcinoma Recurrence to start Carboplatin -Taxol weekly +/- Avastine for 3-4 cycles then re-assess for 2nd look surgery possibility CGS test Including HRD

Signature	Hematology	Pediatric	Bone marrow transplant
dr nada hassan			



MDT Form
[[DOCTYPE:MDT_FORM]]

alfa
SCAN

Name: Doha Mohamed Mahmoud ID: 5178329
Date: 16/05/2026

Multislice post contrast CT scan of the chest, abdomen and pelvis with multiplanar reformatted images revealed:

- ***Given history of recurrent ovarian cancer underwent surgical intervention.***

I- Chest:

- Bilateral upper lung lobes reticulation and scarring.
- Middle lung lobe small pulmonary nodule measuring 3.0 mm in diameter.
- Lingular segment fine atelectatic band.
- No evidence of consolidative patches, cavitary lesions, pulmonary masses or bronchiectatic changes.



- No CT evidence of sizable enlarged hilar or mediastinal lymph node.
- No pleural or pericardial collections seen.
- No gross cardiomegaly.

OPINION:

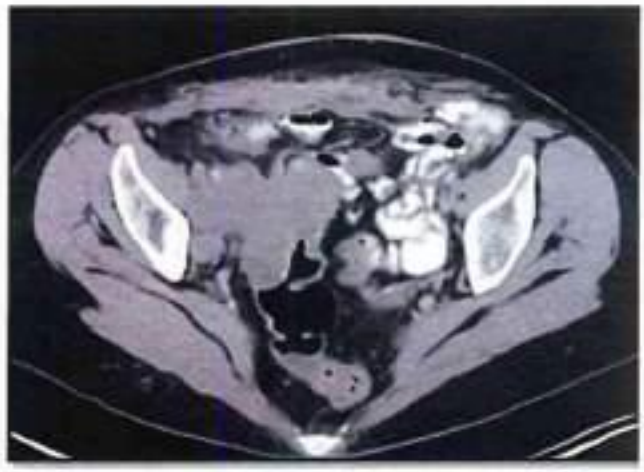
- **Middle lung lobe small pulmonary nodule, for follow up.**

alfa

S C A N

II- Abdomen and pelvis:

- Ill-defined right deep pelvic heterogeneous soft tissue mass lesion measuring about 3.5 x 7.5 cm along their maximum axial dimensions.
    - Such lesion is seen inseparable from/ infiltrating the related segment of the sigmoid bowel loop and seen encroaching upon the lumen. It also seen inseparable from the related ileal bowel loops, which show mural thickening.



    - Partially amalgamated soft tissue lesions (likely nodal) seen inseparable from the aforementioned pelvic lesion. Measuring roughly about 3.0 x 6.0 cm along its maximum axial dimensions. Such lesions seen inseparable from the right external iliac vein and encasing the right iliac external artery.
    - Associated with smudging and stranding of the surrounding fat planes with thickening of the pelvic peritoneal reflections.
- Multiple enlarged right common iliac, right eternal iliac, mesentric, presacral and right inguinal lymph nodes, the largest (right common iliac) measures 1.9 cm in diameter. Other prominent para aortic, aortocaval and left inguinal lymph nodes are noted.
- Right hypochondrial region nodular lesions seen beneath the anterior abdominal wall, the largest measure 1.2 cm in diameter (? Peritoneal).
- Umbilical region nodular isodense lesion is seen measuring 1.2 x 1.4 cm in diameter.
- Enlarged liver showing homogenous parenchymal pattern. No hepatic lesions or biliary radicles dilatation.

ID: 5178329

alfa

S C A N

- Distended gallbladder showing variable sized stones with few of them are marginally calcified.
- Left iliac region incisional hernia is noted with herniated small bowel loop seen insinuated between external and internal oblique muscles.
- Normal CT appearance of the spleen, pancreas, kidneys (apart from right renal upper calyceal tiny stone), suprarenal glands, aorta and IVC.
- Circumaortic left renal vein.
- Lower anterior abdominal wall scarring with smudging and stranding of the subcutaneous fat.
- Distended right common femoral vein. For Doppler correlation.
- No ascites.

OPINION:

- **Right deep pelvic heterogeneous soft tissue mass lesion with adjacent nodal amalgam showing relations as described.**
- **Abdominopelvic lymphadenopathy.**
- **Right hypochondrial region intraabdominal lesions? Peritoneal deposits.**
- **Umbilical region nodular lesion as described.**
- **Left iliac region incisional hernia containing small bowel loops.**
- **Hepatomegaly.**
- **Calcular gallbladder.**
- ***For correlation with intervention data, histopathology and prior studies.***

Dr. Ahmed Zein, MD

Dr. Omnia Abd El Fattah, MD
ID: 5178329

Patient Name	ضحى محمد محمود حسين	**Code Number**	YK 26012427
Sex/Age	Female 42 Y 7 M 24 D	**Patient ID**	90812
Registered Date	25/4/2026 08:01:38 PM	**Referred By Dr.**	شريف عبد الكريم
Validation Date	02/5/2026 06:59:54 PM	**Branch**	المعادي
Reporting Date	04/5/2026 03:51:42 PM		

B) Mild line nodule and nodules: Positive For Soft Tissue Deposits Of Same Described Neoplasm (As A) With Positive Lymph Nodes Deposits (2/7).
C) Omentum: Fibrofatty Tissue: Free Of Neoplastic Deposits.
Tightly Correlate With Clinical, Radiological Findings As Well As Relevant Previous Pathology Reports (If Any).

P.O

Number of slides enclosed: 6

Diagnosed by

Consultant Pathologist
Dr. Rana ElSaeed, MD

معامل الخليـــة
لتحاليل الأنسجة والأورام

Patient Name	ضحى محمد محمود حسين	**Code Number**	YK 26012427
Sex/Age	Female 42 Y 7 M 24 D	**Patient ID**	90812
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Validation Date	02/5/2026 06:59:54 PM	**Branch**	المعادي
Reporting Date	04/5/2026 03:51:42 PM		

Clinical Diagnosis: Left Abdominal Wall Mass (Previous Pan-Hysterectomy For Low Grade Ovarian Cancer)
Nature of the specimen: Excision Biopsy And Omental Biopsy

Pathology Report

Gross Picture

Three designated specimens were received in the same container as:
A) Left abdominal wall mass: Unlabeled fibrofatty muscular tissue piece measured 5x4x2 cm, irregular with ill-defined firm white lesion, totally occupied it, on inked margins.
B) Mild line nodule and nodules: Three nodular tissue pieces measured 1x1 cm, regular measured 4x2.5x2 cm, regular capsulated measured 2x1.5 cm, irregular with firm white cut section.
C) Omentum: Fatty tissue piece measured 12x6.5 cm with no detectable nodule.

Microscopic Picture

A) Sections examined from left abdominal mass showed deposit of papillary malignant neoplasm, tumor forms papillary configuration with luminal necrosis among desmoplastic stroma. The tumor cells showed pleomorphic hyperchromatic nuclei with prominent nucleoli .
B) Sections examined from mid lined nodule and nodules showed deposit of papillary malignant neoplasm, tumor forms papillary configuration with luminal necrosis among desmoplastic stroma. The tumor cells showed pleomorphic hyperchromatic nuclei with prominent nucleoli. Two out of seven lymph nodes showed metastatic deposit of above-mentioned tumor (2/7).
C) Sections of fibrofatty tissue showed free of neoplastic deposits.

Diagnosis

Left Abdominal Wall Mass (Previous Pan-Hysterectomy For Low Grade Ovarian Cancer), Excision Biopsy And Omental Biopsy:
A) Left abdominal wall mass: Positive For Deposits Of Papillary Malignant Neoplasm, For Immunophenotyping.

Scan Me

الفرع الرئيسي الدقي : ٢٠ شارع التحرير أمام مترو البحوث - الدقي - الدور الأول
م: ٠١١١٢٤٤٣٣٣ - ٠١١١١٥٥٧١١٧ - ت: ٠٢٣٣٣٥٣٠٧٣

Pathology Report
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المختبر
al mokhtabar
moamena kamel lab
معامل مؤمنة كامل

PATHOLOGY REPORT

Code:	63923507881/0 -(P 32982-23)			Patient Name:	
Age:	40 Y	Sex:	Female	Specimen Received Date:	09/11/2023
Branch:	Maadi 7 (Street 9)			Date Report Issued:	19/11/2023
Referred By Prof:	شريف احمد عبد الكريم			Unit Manager:	Prof. Dr. Ahmed Abdelaziz

CLINICAL DATA: A case of resected left ovarian cyst, diagnosed as low grade microinvasive carcinoma.
SPECIMEN: Total hysterectomy with bilateral salpingo-oophorectomy, omentectomy and appendectomy.

GROSS PICTURE: Two designated specimens were received:

1-Total hysterectomy with bilateral salpingo-oophorectomy specimen received distorted and weighed 215 gm. Cervix measured 3 cm long. The uterine cavity was 5x3 cm. The endometrium was 0.3 cm. The maximal myometrial thickness was 3 cm. The larger ovary was 4x4x2 cm. The smaller ovary was 3x2.7x1.2 cm. Each oviduct was 8x1.2 cm.

Gross Pathological Findings: **1).** The larger ovary showed sutured cystified rubbery gray white cut section. **2).** The other ovary was grossly unremarkable. **3).** The tubes showed hemorrhagic lining with dilated lumen. There is a separate omental tissue piece measured 10x7 cm with focally congested cut section.

2- Appendix measured 5 cm long & 1 cm in diameter with dull opaque outer surface, inner hyperemic lining & lumen filled with hemorrhagic material.

MICROSCOPIC PICTURE: Serial sections examined ***from the larger ovary*** revealed few foci of invasive tumor tissue formed of tubular and papillary structures with fibrous tissue cores, lined by single to stratified layers of pleomorphic anaplastic cells, with some showing hobnail appearance. The rest of ovarian cyst showed focal lining by endometrial type cells, resting on their stroma, with many siderophages. Scattered foci of inorganic suture-like material was detected. The rest of ovarian tissue showed multiple cystic follicles.
Sections examined from the other ovary revealed multiple cystic follicles, free of tumor deposits. Sections examined from the omental tissue were free of metastatic tumor deposits and showed foci of foreign body giant cell reaction. Sections examined from the cervix revealed chronic non-specific cervicitis. Sections examined from the endometrium revealed secretory changes. Sections examined from the appendix revealed moderate chronic inflammation with lymphoid follicles hyperplasia.

DIAGNOSIS: Total Hysterectomy With Bilateral Salpingo-oophorectomy, Omentectomy and Appendectomy:

- **Larger Ovary: Few Residual Foci Of Microinvasive Papillary Serous Carcinoma, Low Grade.**
- **Free Cervix, Uterine Wall, Other Ovary, Appendix and Omentum.**

**20 enclosed slides.*

Samar

Dr. Samar Amer, M.D.
Assoc Prof. Of Pathology
Faculty of Medicine, Cairo University

NOTE: This report was generated by computer, any manual correction will make it null and void.

PATHOLOGY REPORT

Code:	63923507088 -(P 29760-23)			Patient Name:	ضحى محمد محمود
Age:	40 Y	Sex:	Female	Specimen Received Date:	12/10/2023
Branch:	Maadi 7 (Street 9)			Date Report Issued:	21/10/2023
Referred By Prof:	-			Unit Manager:	Prof. Dr. Ahmed Abdelaziz

CLINICAL DATA: A case of right ovarian cyst, CA125 = 139 U/ml.

SPECIMEN: Right ovarian cystectomy.

GROSS PICTURE: Multiple irregular cyst wall pieces collectively measured 3x3 cm with focally attached and separated papillary brownish growths received fragmented collectively measured 4x4x2 cm.

MICROSCOPIC PICTURE: Sections examined revealed pieces of ovarian cyst wall showing the picture of borderline papillary serous neoplasm. The tumor was lined by papillary structures, with tufting, papillary, micropapillary, cribriform and complex architecture, they were lined by single to stratified layers of columnar cells showed some atypical features, with high N/C ratio, and enlarged mostly vesicular nuclei and prominent nucleoli. The cores of papillae were variable in thickness. Foci of stromal invasion with clusters of cells, rounded aggregates of papillae with eosinophilic cytoplasm and minimal / absent stromal reaction as well as areas of necrosis was detected. Also detected pieces of cyst wall, lined by endometrial type epithelium with underlying endometrial stroma rim as well as sheets of hemosiderin laden macrophages.

DIAGNOSIS: Right Ovarian Cystectomy Specimen:

- **Microinvasive Low Grade Papillary Serous Carcinoma with Endometrioid Differentiation.**

** 4 enclosed slides.*

Dr. Abeer Mohammad Amal, M.D.
Lecturer Of Pathology
Faculty of Medicine, Cairo University

Prof. Dr. Ahmed Abd El-Aziz, M.D.
Professor Of Pathology
Faculty Of Medicine, Cairo University

NOTE: This report was generated by computer, any manual correction will make it null and void.

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Fatth Islamic Center

C.T. - M.R.I

U.S. - Duppler - E.C.G. Stress

E.E.G - E.M.G - Mamm - Eye Unit

الجمعية الشرعية

مركز الفتح الإسلامى

أشعة مقطعية بالكمبيوتر - الرنين المغناطيسى
موجات صوتية - دوبلر ملون - رسم قلب بالمجهود
رسم مخ - رسم عضلات - فحص ثدى - وحدة عيون

Patient Name: ضحى محمد محمود	Study Date: 15/03/2026 01:14:15 PM

M.R.I. OF THE ABDOMEN

Technique:

- *Different pulse sequences in different tissue planes before and after IV contrast administration.*

Findings:

- *The liver is of normal size and regular outline, with no obvious focal mass lesion of abnormal intensity or dilated biliary radicles.*
- *The gall bladder lumen seen harboring multiple fairly equal sized small stones of signal void pattern and each measuring about 1cm in diameter. No mural thickening or peri-cholecystic collections.*
- *The spleen and retroperitoneal structures including the kidneys, adrenal glands and pancreas are within normal limits, with no obvious mass lesion.*
- *No significant lymph node enlargement.*
- *No ascites.*

Conclusion:

- ***Calcular gall bladder.***
- ***Otherwise, essentially normal M.R.I. study of the abdomen.***

YOURS SINCERELY,

Dr. Marwa El-Talkhy, M.D.

ASS. PROF. OF RADIO-DIAGNOSIS

CAIRO UNIVERSITY

aa

٥٣ شارع ١٠ بالمعادي ت : ٢٣٥٨٧٣٣٢ - ٢٣٥٨٥٨١٩ - ٠١١٠٠١٥١٣٨٠ - ٠١٠٢٨٤٨٣٦٩٥

M.R.I
[[DOCTYPE:MRI]]

Fatth Islamic Center

C.T. - M.R.I

U.S. - Duppler - E.C.G. Stress

E.E.G - E.M.G - Mamm - Eye Unit

الجمعية الشرعية

مركز الفتح الإسلامى

أشعة مقطعية بالكمبيوتر - الرنين المغناطيسى
موجات صوتية - دوبلر ملون - رسم قلب بالمجهود
رسم مخ - رسم عضلات - فحص ثدى - وحدة عيون

Patient Name: ضحى محمد محمود	Study Date: 15/03/2026 01:14:15 PM

M.R.I. OF THE PELVIS

Technique:

- *Different pulse sequences in different tissue planes before and after IV contrast administration.*

Findings:



- *Evidence of prior total hysterectomy with intact stump. A fairly defined heterogeneously enhancing soft tissue lesion is seen at the right adnexal region, measuring about 5.5×3.6×4.3cm in its maximum dimensions. The lesion appears inseparable from the adjacent large bowel loops, raising the possibility of bowel adhesion or direct infiltration.*
- *Additionally, enlarged iliac lymph nodes are noted bilaterally, more prominent on the right side. The right iliac lymph node measures approximately 3.8 × 2.6 × 3.8 cm, while a smaller left iliac lymph node measures about 3.0 × 2.3 × 3.5 cm.*
- *The other pelvic organs appear within normal.*
- *No free pelvic collections.*

Conclusion:

- ***Findings are suspicious for a right adnexal neoplastic lesion with associated bilateral iliac lymphadenopathy, more pronounced on the right side. The inseparability from the adjacent bowel loops raises the possibility of local extension or adhesion.***
- ***Further evaluation with histopathological correlation and oncologic assessment is recommended***

YOURS SINCERELY,

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